PREVENTION OF MISCARRIAGE.

The author has met with remarkable success in carrying women beyond critical periods of gestation, which, previous to treatment, had always proved fatal to the fœtus, using the following applications:

First. No. 6 electrode across the lower part of back in E, No. 8 electrode very low on the abdomen, short point downward in C, twenty-five to thirty minutes.

Second. No. 4 electrode on the upper part of the stomach in B, No. 5 electrode between the kidneys on spine in D, ten minutes (to prevent vomiting).

Third. No. 6 electrode across the lower part of the back in A, No. 8 electrode on the abdomen, long point on the stomach in D, twenty-five to thirty minutes. Alternate daily first and third treatment for six weeks before the critical time, and through the month, then twice a week will be sufficient until confinement, unless complicated symptoms arise. When immediate miscarriage seems imminent from some sudden injury, the patient should assume a recumbent position immediately, and suitable electrodes in size and form selected, one applied over the part injured in B, the other above the injury in D, twenty-five minutes or longer, if required. Conclude the treatment with No. 2 electrode at the base of brain in A, No. 6 electrode under the hands in C, five minutes longer, then follow the previous directions.

LABOR ASSISTED.

During the pains No. 5 electrode across upper abdomen in A, the sponge-cup pressed against the